Lomustine + bevacizumab 7.5 mg/kg

Lomustine + bev 7.5 NCCP 00804: recurrent malignant glioblastoma; bevacizumab use is off-label. Use the exact prescription and schedule agreed with your specialist.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · lomustine and bevacizumab

Active medicines: Lomustine + Bevacizumab

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/dose	At least 30 minutes before the single Day1 lomustine dose; record delayed-emesis cover and rescue duration NCCP V6 Table5 p8 and timing p4. Review QT risk, liver function, constipation and interactions. Lomustine is a single dose, not a daily course.

Additional phase-specific support

- Lomustine has moderate-to-high oral emetic potential. Take the prescribed prevention and rescue plan around the single dose. Contact the team if nausea or vomiting prevents fluids or essential medicines; do not automatically repeat a vomited chemotherapy dose.
- Bevacizumab alone has minimal emetic potential. Routine anti-sickness drugs or steroid premedication are not automatically required for every infusion; the team may prescribe specific medicines after a previous reaction or for another reason.
- Keep prescribed blood-pressure medicine and urine checks. Follow the team's plan for any home measurements. Report wounds and upcoming surgery or dental treatment early, so timing and healing can be assessed before another infusion.
- Keep steroids and seizure medicines on their own written schedules if prescribed. Ask what to do after a missed or vomited dose. Other medicines can worsen constipation; agree a bowel plan and have all new medicines checked for interactions.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00804 v2 · p5 · NCCP oral-SACT antiemetics V6: Table5 p8 and timing p4

Sources checked 2026-11-05 · SACTCheck v0.84.39 · Regimen-level drafts; prescriber and oncology-pharmacy verification required